

Clinical Study Summary Report (CSSR)

Document Status: Final | Version: 1.0 | Date: 01-APR-2026

1. Administrative & Study Identification

Full Study Title:	A Single-arm Multi-center Study of Ruxolitinib in Chinese Participants With Corticosteroid-refractory Chronic Graft Versus Host Disease After Allogeneic Stem Cell Transplantation
Short Title:	Study of Efficacy and Safety of Ruxolitinib in Chinese Participants With Corticosteroid-refractory Chronic Graft vs. Host Disease
Protocol Number:	486827818258007381
NCT Number:	NCT06824103
Sponsor Name:	Novartis
Investigational Product:	ruxolitinib
Phase:	PHASE4
Medical Monitor:	[Not Provided in Posting]

2. Study Synopsis & Rationale

2.1 Study Objective

Primary Objective:	1. Overall Response Rate (ORR): ORR is defined as the percentage of participants demonstrating a complete response (CR) or partial response (PR) without the requirement of additional systemic therapies for an earlier progression, mixed response or non-response, according to National Institute of Health (NIH) Consensus Criteria.
Secondary Obj.:	1. Failure-free Survival (FFS) 2. Best Overall Response (BOR) 3. ORR at end of cycle 3

2.2 Background & Rationale

The purpose of the study is to assess the efficacy and safety of ruxolitinib in Chinese adult and pediatric participants aged 12 years or older with corticosteroid-refractory chronic graft vs. host disease (SR-cGvHD).

3. Study Design & Methodology

Design Framework:	Type: INTERVENTIONAL, Phase: PHASE4, Status: RECRUITING
Target N:	50

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria

- * Signed informed consent must be obtained prior to participation in the study.
- * Male or female Chinese participants aged 12 or older at the time of informed consent
- * Able to swallow tablets.- Have undergone alloSCT from any donor source (matched unrelated donor, sibling, haplo-identical) using bone marrow, peripheral blood stem cells, or cord blood. Recipients of non-myeloablative, myeloablative, and reduced intensity conditioning are eligible.
- * Evident myeloid and platelet engraftment:

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- * Absolute neutrophil count (ANC) $\geq 1,000/\text{mm}^3$ AND
- * Platelet count $\geq 25,000/\text{mm}^3$

Note: Use of growth factor supplementation and transfusion support is allowed during the trial, however, transfusion to reach a minimum platelet count for inclusion is not allowed during screening and at baseline.

- * Participants with clinically diagnosed cGvHD staging of moderate to severe according to NIH Consensus Criteria (Jagasia et al 2015) prior to Cycle 1 Day 1.

- * Moderate cGvHD: at least one organ (not lung) with a score of 2, 3 or more organs involved with a score of 1 in each organ, or lung score of 1.

- * Severe cGvHD: at least 1 organ with a score of 3, or lung score of 2 or 3.

- * Participants currently receiving systemic corticosteroids for the treatment of cGvHD for a duration of ≤ 12 months prior to Cycle 1 Day 1, and have a confirmed diagnosis of corticosteroid refractory cGvHD defined per 2014 NIH consensus criteria (Martin et al 2015) irrespective of the concomitant use of a calcineurin inhibitor, as follows:

- * A lack of response or disease progression after administration of minimum prednisone 1 mg/kg/day for at least 1 week (or equivalent) OR

- * Disease persistence without improvement despite continued treatment with prednisone at ≥ 0.5 mg/kg/day or 1 mg/kg/every other day for at least 4 weeks (or equivalent) OR

- * Increase to prednisone dose to ≥ 0.25 mg/kg/day after two unsuccessful attempts to taper the dose (or equivalent)

- * Participants has Eastern Cooperative Oncology Group (ECOG) performance status of 0-2

4.2 Exclusion Criteria

For a full list of exclusion criteria, refer to Section 5.2. Key exclusion criteria include

- * Participants who have received two or more systemic treatments for cGvHD in addition to corticosteroids $\pm$ CNi for cGvHD.

- * Participants who have received ROCK2 inhibitors for cGvHD.

- * Participants that transition from active aGvHD to cGvHD without tapering off corticosteroids $\pm$ CNi and any systemic treatment

Note: Participants receiving up to 30 mg by mouth once a day of hydrocortisone (i.e., physiologic replacement dose) of corticosteroids are allowed.

- * Participants who were treated with prior JAK inhibitors for aGvHD; except when the participant achieved complete or partial response and has been off JAK inhibitor treatment for at least 8 weeks prior to Cycle 1 Day 1.

- * Failed prior alloSCT within the past 6 months from Cycle 1 Day 1.

- * Participants with relapsed primary malignancy, or who have been treated for relapse after the alloSCT was performed.

- * SR-cGvHD occurring after a non-scheduled donor lymphocyte infusion (DLI) administered for pre-emptive treatment of

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malignancy recurrence. Participants who have received a scheduled DLI as part of their transplant procedure and not for management of malignancy relapse are eligible.

Other protocol-defined inclusion/exclusion may apply.

11. Study Identifiers & Governance

Primary ID:	486827818258007381
Registry Number:	NCT06824103
Sponsor:	Novartis
Study Title:	Study of Efficacy and Safety of Ruxolitinib in Chinese Participants With Corticosteroid-refractory Chronic Graft vs. Host Disease

15. Sign-off & Approval

Principal Investigator:	_____	Date:	_____
Clinical Operations Lead:	_____	Date:	_____
Lead Statistician:	_____	Date:	_____